

Standard Operating Procedure

Patient Transfer (In, Out) and Referral

«Hospital Name» — Supports AAC.3.a, AAC.3.b, AAC.3.c, AAC.3.d

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» receives transfer-in patients, transfers out or refers patients to another facility, assigns appropriate accompanying staff, and provides a summary of condition and treatment — safely, every time.

2. Scope

Applies to emergency, every ward, ICU, day-care, and any unit that receives a transfer-in or initiates a transfer-out, referral, or diagnostic shift outside the immediate unit.

3. Responsibility

Treating Doctor: decides transfer need, ensures stabilisation, judges stability, and authorises transfer-out or referral.

Accompanying Staff (nurse/technologist): must hold at least basic or advanced CPR training as appropriate to the patient's condition.

Emergency In-Charge: coordinates transfer-in arrangements including any escort sent with the ambulance.

Medical Records: verifies the retained summary copy is filed under the unique identification number.

4. Procedure

4.1 Transfer-in (planned and unplanned)

1. For unplanned transfers or suspected unstable patients, consider sending a suitably trained person with the ambulance, guided by the information received from the referring facility.
2. On arrival, register the patient under the hospital's registration process. The receiving doctor documents clinical status on arrival and whether the transfer was planned or unplanned.
3. Send feedback to the referring organisation or doctor where appropriate.

4.2 Transfer-out and referral

1. Consult the patient and/or family before transfer, where they can be consulted.
2. Address pre-transfer stabilisation, choice of mode/vehicle, equipment required, and monitoring required — document each element. This follows the ACEP policy on appropriate interfacility transfer — see Section 9, Reference 1.
3. If any stated requirement cannot be met, document the reason in the transfer record before the patient leaves — never leave it undocumented.
4. The treating doctor authorises every transfer-out or referral.

4.3 Assigning accompanying staff

1. Assign staff who hold at minimum trained trauma technologist, emergency technologist, or nurse status, with basic or advanced CPR training as appropriate to the patient's condition. "Basic" and "advanced" CPR training correspond to the American Heart Association's Basic Life Support (BLS) and Advanced Cardiovascular Life Support (ACLS) certification levels, or the hospital's equivalent recognised training.
2. A doctor must accompany any unstable admitted patient being transferred out or shifted for diagnostics. Judge stability against airway, breathing, circulation, and conscious level, per the written stability criteria.
3. Record the names and roles of accompanying staff on the transfer summary.

4.4 Transfer/discharge summary

1. Give a transfer summary stating significant findings and treatment given, to every patient transferred from emergency or shifted for diagnostic or therapeutic purposes.
2. If the patient is discharged and transferred out — including leaving against medical advice (LAMA) — give a discharge summary instead, following the same standard.
3. Retain a copy of every transfer or discharge summary. Medical Records files it under the unique identification number before closing the episode.

5. Stop-Work Authority

Do not transfer an unstable patient out, or shift an unstable admitted patient for diagnostics, unless transfer criteria are met, pre-transfer stabilisation is done, and appropriate accompanying staff are assigned.

If those conditions are not met: stop the transfer, continue stabilisation, and escalate to the treating doctor and the Medical Superintendent the same shift. Document why the transfer was held.

Stop-work never blocks emergency life-saving measures — it only blocks unsafe transfer movement. Refusing an unsafe transfer is not a disciplinary matter.

6. Records

Supported by all 16 records in the AAC.3 Required Records pack, covering transfer-in, transfer-out/referral, accompanying staff, and transfer/discharge summaries.

7. Related Documents

- HCO_AAC_3_v2_REWRITE_DRAFT — AAC.3 policy
- SOP — Patient Registration, Identification and Admission

8. Review

Reviewed annually, and sooner after any adverse event during a transfer.

9. References

- American College of Emergency Physicians. (2022). Appropriate interfacility patient transfer (Policy statement). <https://www.acep.org/patient-care/policy-statements/appropriate-interfacility-patient-transfer/>
- American Heart Association. (n.d.). Basic Life Support (BLS) and Advanced Cardiovascular Life Support (ACLS) courses. <https://cpr.heart.org/en/cpr-courses-and-kits/healthcare-professional>